

EXECUTIVE BRIEF

Quality of Care Is Not Activity Alone

What leaders should authorize, require, and refuse

Production Unit	PU-B07-C31
Decision status	Controlled learning asset
Audience	Executives, programme owners, statisticians, technical leads
Data	Synthetic examples only

Executive takeaway: Measure quality of care through outcomes, safety, experience, equity, continuity, and clinical standards—not activity alone.

Why it matters

High activity can coexist with unsafe, inequitable, ineffective, or fragmented care.

Illustrative application

A facility combines service volume with outcomes, adverse events, continuity, and patient experience.

Minimum approval conditions

- Named accountable owner and lawful purpose
- Defined evidence, provenance, reference period, population, and geography
- Decision-relevant validation and visible uncertainty
- Documented abstention, referral, override, and incident processes
- Accessibility, equity, security, retention, and retirement review

Non-negotiable boundary

The analytical system may inform review. It may not independently authorize benefits, sanctions, enforcement, clinical treatment, school exclusion, displacement, or another consequential action.